

Brain-Based Mental Health Community

Play Therapy Feedback

NAME

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DATE

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Review the live website before providing feedback:

<https://tcfasd.github.io/Brain-Based-Mental-Health-/handout-therapy-guide.html>

Includes: Child-Centered Play Therapy (CCPT), Directive Play Therapy, Filial Therapy Ages 2–12+ | Brain-Based Fit: Strong

What It Asks the Brain to Do:

Play therapy works through the experiential and emotional brain — not through words or insight. Play is the natural language of childhood, and children under 10 typically lack the cognitive development for abstract talk therapy.

- **Child-Centered (CCPT):** The child leads; the therapist follows, reflects emotions, and provides unconditional positive regard. The brain expresses and processes emotions through play rather than words.
- **Directive:** The therapist introduces specific activities (sand tray, art, puppets, structured games) to address identified issues. The brain engages with specific content through accessible, concrete means.
- **Filial Therapy:** The therapist trains the caregiver to conduct child-centered play sessions. The caregiver learns to follow the child's lead, reflect emotions, and set limits — strengthening the attachment bond through play.

Caregiver Involvement:

Varies dramatically by type. CCPT traditionally has low caregiver involvement (the therapy happens between child and therapist). Filial Therapy has very high caregiver involvement — the caregiver becomes the therapeutic agent. If caregiver involvement matters to you (and for brain-based differences, it should), ask specifically about Filial Therapy.

How to Know If It Might Be a Good Fit:

- Your child is young or has developmental delays that make talk therapy difficult
- Your child communicates better through action and play than through words
- Emotional expression, processing, or attachment is the primary concern
- Previous talk-based therapies haven't connected with your child

Brain-Based Consideration

:Play therapy may be the most accessible individual therapy modality for children with brain-based differences because it works through the experiential brain, bypasses verbal and cognitive demands, and meets children at their developmental (not chronological) level. Filial Therapy is especially promising because it strengthens the caregiver relationship and teaches the caregiver to attune to the child's actual developmental level.

1. Is what is written accurate?



Yes



No

Yes

2. Is there additional information that you think caregivers should know about Play Therapy?

Thank you for your feedback!

Play therapy accesses the right side of the brain (art, music, expression, color) while more traditional talk therapy considers only the left side of the brain (language, math, organization, black and white). Since the brain grows from right to left and bottom to top, play therapy is perfect for children, especially those who show dysmaturity in various areas, causing their emotional or other developmental age to lag behind or possibly even far exceed their chronological age. It is effective also for older children, teens, and even adults when implemented sensitively, with credit to neurointegration, or the way play therapy works to process information through both sides of the brain. It's extremely effective when a more "front door" approach doesn't seem to work.